

SUBJECTIVE:

- Patient reports acute onset chest pain, stating, "I have a chest pain recommend me some medicine."
- Describes the pain as a severe, substernal pressure sensation, radiating to the left arm and jaw.
- Pain started approximately 2 hours prior to presentation and has been constant.
- Rates pain as 7 out of 10 in severity.
- Associated symptoms include mild shortness of breath, diaphoresis, and nausea.
- Denies recent trauma, fever, cough, or recent strenuous activity.
- Expresses a desire for immediate pain relief medication.
- No known cardiac history or previous episodes of similar chest pain.

OBJECTIVE:

- Expected Physical Exam Findings:
- General: Patient appears anxious and in acute distress, pale and diaphoretic.
- Cardiovascular: Expected regular heart rhythm, possibly with mild tachycardia. No expected murmurs, rubs, or gallops on initial auscultation, though a new S3 or S4 could indicate cardiac dysfunction.
- Respiratory: Lungs expected to be clear to auscultation bilaterally, though mild tachypnea may be present. No adventitious sounds such as wheezes or crackles.
- Abdominal: Soft, non-tender, non-distended, no hepatosplenomegaly.
- Extremities: No peripheral edema or cyanosis.
- Vital Signs:
- Vital signs are currently unavailable. It is crucial to obtain immediate measurement of heart rate, blood pressure (bilateral), respiratory rate, oxygen saturation, and temperature to assess hemodynamic stability.

ASSESSMENT:

- Primary Diagnosis: Chest Pain, High Suspicion for Acute Coronary Syndrome (ACS)
- Differential Diagnoses:
- Acute Myocardial Infarction (AMI) / Unstable Angina
- Aortic Dissection
- Pulmonary Embolism (PE)
- Pericarditis / Myocarditis
- Pneumothorax
- Esophageal Spasm / Gastroesophageal Reflux Disease (GERD)
- Musculoskeletal Chest Pain
- Anxiety/Panic Attack

PLAN:

- Tests:
- Immediate 12-lead Electrocardiogram (ECG) to assess for ischemic changes.
- Serial cardiac biomarkers (Troponin I/T, CK-MB) to rule out myocardial injury.
- Complete Blood Count (CBC), Complete Metabolic Panel (CMP).
- Chest X-ray to evaluate for pulmonary pathology or aortic widening.
- D-dimer if Pulmonary Embolism is a significant concern.
- Consider bedside echocardiogram if available and clinically indicated.
- Medications:
- Aspirin 325mg chewable STAT (unless contraindicated).
- Sublingual Nitroglycerin 0.4mg every 5 minutes for up to 3 doses (if blood pressure allows).
- Oxygen supplementation via nasal cannula if SpO2 < 94% or patient is in respiratory distress.
- IV access and IV fluids as needed for hydration or medication administration.
- Morphine IV for pain relief if unresponsive to nitroglycerin and hemodynamically stable.
- Beta-blockers (oral or IV) if no contraindications, especially if tachycardia or hypertension is present.
- Patient advice:
- Remain calm and avoid any physical exertion.
- Report any changes in pain or associated symptoms immediately.
- Understand the urgency of prompt diagnosis and treatment.
- Follow-up instructions:

- Immediate transfer to an Emergency Department for comprehensive evaluation, monitoring, and management.
- Based on initial findings, potential admission to a cardiac intensive care unit or cardiology service.
- Follow-up with a cardiologist will be arranged upon discharge for further risk stratification and management.